جوستاف روسي انترناشيونال - مصر (EPE) *

طلب علاج أول مرة / متردد

إسم المريض رباعياً : شيماء سعد دسوقي ابراهيم
المحافظة : الجيزه **الحي / المدينة :** البدرشين **العنوان :** قريه دهشور
رقم البطاقة : 29411052103621 **رقم التليفون :** 01116270023 **تاريخ تسجيل الإستمارة :** 14-10-2026
العمر(تاريخ الميلاد) : 05-11-1994 **المهنة :** ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي **المقر بما فيه**
التوقيع:(----------) **التوقيع:**(----------)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : علاج مبادرة سرطان الثدي زولاديكس تحت الجلد لمده 6 شهور [14163] **قسم العلاج :** الاورام
التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------
خطة العلاج : ----------
المستشفي : ----------**قسم :** ----------
جهة العلاج : ----------**نفقات :** ----------
ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: شيماء سعد دسوقي ابراهيم
National ID: 29411052103621

GUSTAVE ROUSSY INTERNATIONAL EGYPT

تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي Goserelin (Zoladex) subcutaneous

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين





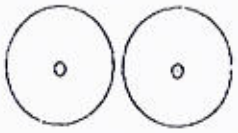
National ID · Front & Back
[[DOCTYPE:ID_FRONT_BACK]]

Presidential Program for Women's Health
Ministry of Health and Population

البرنامج الرئاسي لصحة المرأة
وزارة الصحة والسكان

MDT form

A-Patien: ID:
1- Name : شيماء صبحه بسوقي ابراهيم
Sex: Female
3- Mecical number: 83220714
Old Med : 2348-2024
3-ID number:
Patient Age : 30 Y



Family history	-ve	Phone	01116270923	BMI		REASON OF MDT	
marital status		NRS Score		GROUP		Date of MDT	01/02/2025
Co morbidity	-ve	Menopausal status		Initial STAGE			

Clinical data

Breast Imaging :
rt locally advanced with pathological LN UIQ 7.1 -6.5 x8.3

Mets Workup :
free

BX	ER	PR	Her 2 Neu	SISH	Ki67
IDC	7/8	0/8	3		32

T	N	M
4b	1	0

Sonomammography

PATIENT HISTORY

received NACT 6 TCHP +5 HP
RT SM+SLNB: IDC GII
FREE deep margin , nipple ,a reola
LN 0/3 (frozen
LN one show micrometastattic <0.2 mm other two (0/2)

MDT OUTCOME & PLAN

FOR KADCYLA UP TO 1 YEAR
ZOLADEX - TAM Arimidex as pt is Right Risk 30%. cTub, N1
PORT Rt SSM 4pT2

SIGNATURE

MEDICAL ONCOLOGY		RADIOLOGY	
SURGERY		PATHOLOGY	
Psychiatric		Nutrition	

MDT Form
[[DOCTYPE:MDT_FORM]]

Name: شيماء سعد دسوقى ابراهيم	
MR. : 5581	

Detailed Medical Report

السيد	

تحية طيبة وبعد,,

إيماء للطلب المقدم من		**بشأن تقرير طبي للمريض**	
بقسم:		**رقم طبي:**	

ننشرف بأن نوضح فيما يلي تقريرا عن حالتها

تاريخ وساعة الدخول :		**تاريخ وساعة الخروج :**	

وبعد الكشف والفحوصات والمتابعة وجد انها

The patient is a 30-year-old with a prior right-sided breast cancer, clinical stage T4b N1, invasive ductal carcinoma, estrogen-receptor (ER) 7/8 positive, progesterone-receptor (PR) negative, HER2 3+ positive. The disease was treated with neoadjuvant chemotherapy (TCHP) completed on 25 December 2024, followed by radiation therapy (RT) to the breast and supraclavicular nodes. Pathology after surgery showed residual disease (REC) with a 1/2 positive tumor (1/2PT). The patient subsequently received adjuvant KADCYLA, the last dose administered on 20 January 2025, and ZOLADEX. Because the patient is considered high-risk and tamoxifen (TAM) is unavailable, the plan was to shift to ARIMIDEX. Imaging in October 2025 demonstrated a free left femoral lesion and a free left sacroiliac joint; MRI of the left sacroiliac joint was scheduled for September 2025.

The patient completed adjuvant KADCYLA and, due to high-risk status, the regimen was changed to ARIMIDEX with addition of ZOMETA. The note indicates that the triple-positive breast cancer treatment was finished and the patient was to continue ARIMIDEX and ZOMETA.

On the subsequent visit, the patient was receiving ZOMETA, ARIMIDEX, and ZOLADEX, with pending supportive laboratory studies. The ordering physician requested addition of paracetamol as needed for pain control (<3 g per day).

The patient continued on ZOLADEX and ARIMIDEX, with a prescription for CAL ONE ALFA directed to the pharmacy for the next two months. A follow-up visit was scheduled after three months.

The patient remains on an aromatase inhibitor (AI) regimen with ZOLADEX. Imaging in June 2026 showed a fatty liver on lipid profile and liver function tests, and the disease was noted as free of metastasis (SM free) at that time.

The most recent assessment notes that a sacroiliac joint MRI performed in July 2026 was normal, and prior imaging in June 2026 confirmed no metastatic disease and fatty liver on ultrasound. The patient continues on ARIMIDEX and ZOLADEX with supportive care, and the next follow-up is planned for 1 November 2026.

تحريرا في ,

التاريخ :	21-09-2026	**الوقت :**	
الطبيب المعالج :	هنا اسامه عبد الرحيم عبد البر		

يعتمد ,

USTAVE / OUSSY
INTERNATIONAL

Patient Name	: الأستاذه / شيماء سعد دسوقى ابراهيم		
Patient No.	: 29411052103621	Request No.	: 13008593
Path No.	: 5186/24	Request Date	: 25/12/2024
Sex	: Female	Verification Date	: 19/01/2025
Age	: 30 Y	Financial Category	: جوستاف روسى - مباشره
Referreing Doc	:	Department	: Out Patient
		Registered by	: Mrs.Mayada Hegazy

Pathology Report

Clinical Data: Right breast cancer, post NACT.
Nature Specimen: Right simple mastectomy and SLNs.

Gross Examination:
*Right SLN: A fibro fatty tissue piece measured 4x3 cm entangling 3 nodules.
* Procedure: Right simple mastectomy specimen measured 24x18x5 cm with covering skin ellipse measured 22x10 cm.
* Tumor extent: A rubbery greyish white tumor bed site.
* Size: 8x7x3cm
* Skin involvement: Nipple & areola were unremarkable grossly.
* Distance from margins: 2 cm away from deep margin.
Frozen section result:
SLNs (0/3).

Microscopic Examination:
* Histological type: Invasive duct carcinoma. (cellularity 15%).
* Grade (differentiation+ nuclear pleomorphism+ mitosis): Grade II (3+2+1)
* Angioinvasion: No.
* Ductal carcinoma in situ: Positive 2%.
* Skin involvement: Free nipple and areola.
* Lymph node involvement: One lymph node show micrometastatic deposit < 0.2 mm, other two lymph nodes are negative (0/2).
* Resection Margins: Free deep margin.
* Histologic response to therapy: Partial pathological response to therapy.
* Microcalcifications: Undetected.
* Additional pathologic findings: Hyalinosis, fibrosis (effect of therapy).

Diagnosis:

Verified by :
Dr.Toka Medhat

غرفة رقم (4) – الدور الاول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

GUSTAVE ROUSSY

Patient Name	: الأستاذه / شيماء سعد دسوقى ابراهيم	Request No.	: 13008593
Patient No.	: 29411052103621	Request Date	: 25/12/2024
Path No.	: 5186/24	Verification Date	: 19/01/2025
Sex	: Female	Financial Category	: جوستاف روسى - مباشره
Age	: 30 Y	Department	: Out Patient
Referreing Doc :		Registered by	: Mrs.Mayada Hegazy

Pathology Report

Right breast cancer, post NACT, simple mastectomy and SLNs Biopsy:
Invasive duct carcinoma, Grade II, with DCIS.
Actual tumor cellularity: 15%.
Free deep margin.
Free nipple and areola.
One lymph node show micrometastatic deposit < 0.2 mm, other two lymph nodes are negative (0/2).
Partial pathological response to therapy.
Stage: ypT2, ypN1mi
RCB: 2.865, RCB class-II .
*Comment : There is disparity between frozen and paraffin results , However the final diagnosis depend on the paraffin results due to error in block quality.

Signature
Dr.Salma Tarek.
Dr.Samia Ali.

Verified by :
Dr.Toka Medhat

غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

ECHO REPORT

Name:

Date: 3 / 9 / 2025

EF	63%	55-75%	PWT	0.9	0.6-1.1cm
LVEDD	4.5	3.5-5.8cm	AO.R	2.6	2-3.7cm
LVESD	2.6	2-4cm	LA	3.8	2-4cm
FS	30%	25-45%	RV		0.7-2.3com(supine)
SWT	0.8	0.6-1.1cm	AO.CUSP		1.5-2.6cm

LV dimensions Normal

LV wall thickness Normal

LV diastolic function Normal

SWMA No RWMA

LA and Ao root dimensions Normal

Mitral Valve

Aortic valve N

Tricuspid valve

Pulmonary valve N

Right side and systolic function TAPSE: 2.6

Intracardiac masses and effusion

Dr/

Ministry Of Health
Specialized Medical Centers
Dar El Salam Cancer Hospital
Radiology Department

وزارة الصحة والسكان
أمانة المراكز الطبية المتخصصة
مستشفى أورام دار السلام
ادارة الاشعة

Patient Name: Shaimaa Saad Desouki	Date : 12/12/2024
Exam : ultrasound BREAST (BOTH SIDE)	Age : 30 Year(s)
Medical ID: 2348/2024	Pat. ID: 76686

Combined right breast mammography and complementary ultrasound

Indication:

Right breast pathgologically proven malignant mass on NACT for assessment.

Technique

- Right MLO views
- Right breast ultrasound

Report:

- Scattered fibro glandular density (ACR B)
- Right breast shows UOQ amorphus and pleomorphic calcifications, of segmental distribution, measured 8x11cm with related anterior third distortion, more appreciated in 3D tomosynthesis.
- Complementary ultrasound revealed right breast area of altered parenchyma, with distortion, seen at 12 to 1 o'clcok para-areolar, measured 1.2x3.6cm
- Right axillary LN, seen with diffuse cortical thickness, reaching 4mm
- Free right infra and supra-clavicular regions.

Opinion:

- Partial radiological response of the right breast UOQ malignant mass and axillary LNs (BIRADS 6)

Yours truly

Signature Dr. Engy Abdallah

12/12/2024 3:04 PM

Mammogram
[[DOCTYPE:MAMMOGRAM]]

PATIENT NAME: SHAIMAA SAAD DESOKY IBRAHIM
GENDER: Female
DATE : 19-Aug-2024 02:40 PM

PATIENT ID: 10124247717
AGE: 29 y
ORDER: 2996732

99MTC-MDP BONE SCAN

Desired Information: -
skeletal survey.

Technique:
Delayed Whole body imaging in anterior and posterior projection performed 2 ours after I.V administration of 20 mCi 99m Tc-MDP in the following manner:

Report:

c hot spot that account for osseous deposits could be detected allover the rveyed skeleton.
dd degenerative changes are seen over lumbar vertebrae.
oth kidneys are normally visualized.

Conclusion:

- definite evidence of metastatic osseous deposits.

Reported By,

Dr. Mada Fathy, Nuclear Medicine Consultant 19-Aug-2024 02:34 PM

Bone Scan
[[DOCTYPE:BONE_SCAN]]

Ministry Of Health
Specialized Medical Centers
Dar El Salam Cancer Hospital
Radiology Department

وزارة الصحة والسكان
أمانة المراكز الطبية المتخصصة
مستشفى أورام دار السلام
ادارة الاشعة

Patient Name: Shaimaa Saad Desouki	Date : 23/07/2024
Exam : C.T ABDOMEN &PELVIS	Age : 29 Year(s)
Medical ID: 2348/2024	Pat. ID: 76686

NON-CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN AND PELVIS:

- **Reported case of breast cancer.**

I Chest:

- *Prominent bilateral axillary (level I & II) lymph nodes, largets is seen at right level I measures about 1.5 x 1 cm.*
- No sizable pulmonary nodules, masses, areas of ground glass attenuation, consolidation or cavitary lesions.
- No CT evidence of pathological mediastinal lymph node enlargement.
- No pleural or pericardial collection, pleural thickening or masses.
- No cardiac gross abnormalities.

II Abdomen & pelvis:

- The liver is enlarged showing homogenous parenchymal attenaution. No definite focal lesions on non-conrast basis.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal non contrast CT appearance of the spleen, pancreas, kidneys and suprarenals glands.
- Averagely filled urinary bladder showing no sizable diverticular outpouching.
- Prominent para-aortic · meseneruci and ileo-colic lymph nodes. The largest one appears ileo-colic measures about 1.4 x 1 cm.
- IUCD is noted.
- No ascites.

III The Examined Skeleton:

- Left femoral intertrochanteric small well-defined subcortical lytic lesion with sclerotic outline with no cortical breaching or extra ossoeus soft tissue component.

CT Scan
[[DOCTYPE:CT_SCAN]]

MDT form

3- Medical number: 33220714
Old Med :
Female
Patient Age : 30 Y
2348-2024

Phone 01116270023
NRS Score
Menopausal status
UH1
GROUP
Initial STAGE
REASON OF MDT
Date of MDT 01/02/2025

Clinical data

Imaging :
Presented with pathological LN UIQ 7.1 x 6.5 x 6.3
Mets Workup :
Free

ER	PR	Her 2 Neu	SISH	Ki67
7/8	0/8	3		12

N	M
	0

Mammography

UNT HISTORY
NACT 6 TCHP +5 RP
SLNB: IDC G11
Clear margin , nipple ,a reola
(frozen
show micrometastattic <0.2 mm other two (0/2)

MDT OUTCOME & PLAN

KADCYLA UP TO 1 YEAR
RT - TAM

SIGNATURE

MEDICAL ONCOLOGY		RADIOLOGY	
SURGERY		PATHOLOGY	
Endocrine		Nutrition	

MDT Form
[[DOCTYPE:MDT_FORM]]